

LIVE

Learning Space

NEWSPAPER 29th OCTOBER, 2018

ITEM WISE EDITORIALS

44th WEEK



UHC ... PHILIPPINES EXAMPLE

UPSC CSE – GS II

GS II ... Social Justice and Governance



Learning Space

LIVE



NEWSPAPER EDITORIALS

... DISCUSSION

29th OCTOBER, 2018

6

Lessons from the Philippines



Illustration: Arjun Arora

A secondary health care system has not kept pace with the growth of health care in India. The world's largest public health-care system promises to cover 400 million poor and vulnerable people, but will it have a transformative impact in a country where this is defined mostly by the private sector? Universal care is not just a matter of accessibility and affordability but also quality as India prepares to fight the emerging patterns of chronic non-communicable diseases.

An initiative, who was involved in the implementation of the Universal Health Coverage (UHC) in my own country, the Philippines, with a population of 100 million people, 110 million to be precise, is a perspective on some of the challenges.

Health expenditure

The first is the implementation of robust financial systems in health care. How does the government spend money on the health of its people?

In the Philippines, we went from a negligible government spending on health by financing it through an income tax and alcohol. After additional taxes on tobacco and alcohol, the government increased health care expenditure from 1.5% to 4.5% of gross domestic spending. These extra taxes added to tobacco and alcohol were "embedded" in health by legislation. The tax law allocated 85% of the additional revenue to the health care system and the remaining 15% was allocated to tobacco farmers, who would be affected by the higher tobacco prices.

This is a unique approach and the first in the world where the law allocated funds to public health, specifically towards UHC. The opportunity to health-care financing led to larger health expenditure, which then led to a modernisation of thousands of public hospitals in the Philippines.

For India, a better health care spend will not only make it accessible to all but also enable the government to procure advanced medical technology from around the world to meet the Indian market.

As India grows economically and socially, the shift in the disease burden is becoming more pronounced. Today, non-communicable diseases are posing a challenge to health-care services in the country. The government must recognise the gap in the health system and bridge them to make modern and equitable services available to the poor. There is a need for primary health centres in all parts of the country to adapt to the changing disease profile. For instance, in the past, primary care meant prevention through better sanitation and hygiene in the homes of those. The methodology required immunisation, oral rehydration and local medicine. But because the disease burden has shifted, primary care has changed. Today, primary care involves weight management, blood pressure monitoring, blood sugar monitoring, ECG, blood chemistry, and others. To tackle this demand, India needs modern technology, personal monitoring devices, telemedicine, data analysis and predictive care of it. There is an evident need to provide these services with modern laboratory equipment for blood tests, ultrasound, radiology, ECG, endoscopy, biopsy, digital x-ray CT scanners, etc.

Towards quality

The next challenge is in providing quality health care with a focus on patient safety. The government needs to invest in super hospitals that are needed to bridge the most innovative and effective medical technologies in India. They may seem expensive as an investment, but in the long run will be cost-effective.

Global technological advancements coming to India also require trained medical personnel such as radiologists, oncologists, physical therapists, mental health professionals, clinical nurses, health information specialists and more, more to address the modern management of these lifestyle diseases. Although we are focusing, respectively, data analysis and health systems, we are overlooking the role of the UHC implementation in India. The most important tool for enhancing the UHC is to provide financial risk protection and ensure health security. There are gaps that medical innovations can fill. The world is now watching India's experience with it.

THE HINDU
INDIA'S NATIONAL NEWSPAPER SINCE 1878

14th October, 2018

6 Lessons from the Philippines

A revolutionary journey in India's health-care system has just begun with the launch of Ayushman Bharat. The world's largest public health-care scheme promises to cover 100 million poor and vulnerable families. But will it have a transformative impact in a country where this is delivered mostly by the private sector? Medical care is not just a matter of accessibility and affordability but also quality as India prepares to fight the emerging problem of chronic non-communicable diseases.

As someone who was involved in the implementation of the Universal Health Coverage (UHC) in my own country, the Philippines, with a population of 110 million people, I'd like to share a perspective on some of the challenges.

Health expenditure

The first is the implementation of robust financial reforms in health care. How does the government spend more on the health of its people?

In the Philippines, we were able to quintuple government spending on health by financing it through sin taxes for tobacco and alcohol. After additional taxes on tobacco and alcohol, the government increased health-care expenditure from 1.8% to 4.8% of government spending. These excise taxes added to tobacco and alcohol were "attributed" to health by legislation. The Sin Tax law attributed 85% of the additional excise tax revenues to the health-care system and the remaining 15% was attributed to tobacco farmers, who would be affected by the higher tobacco prices.

This is a unique approach and the first in the world wherein the law attributed funds to public health, specifically towards UHC. The apportioning to health-care financing led to larger health expenditure, which then led to a modernisation of thousands of public hospitals in the Philippines.

6 Lessons from the Philippines

For India, a better health-care spend will not only make it accessible to all but also enable the government to procure advanced medical technology from around the world to enter the Indian market.

As India grows economically and socially, the shift in the disease burden is becoming more pronounced. Today, non-communicable diseases are posing a challenge to health-care services in the country. The government must recognise the gaps in the health system and bridge them to make modern and equitable services accessible to the poor. There is a need for primary health centres in all parts of the country to adapt to the changing disease profile. For instance, in the past, primary care meant prevention through better sanitation and hygiene as the burden of illness.

The methodology required immunisation, oral rehydration and hand washing. But because the disease burden has shifted, primary care has changed too. Today, primary care involves weight management, blood pressure monitoring, blood sugar monitoring, ECG, blood chemistry, and others. To tackle this demand, India needs modern technology, personal monitoring devices, telemedicine, data analytics and extensive use of IT.

There is an evident need to provide these centres with modern laboratory equipment for blood tests. Ultrasound, radiology, ECG, endoscopy, laparoscopy, maybe even CT scanners, are needed.

Towards quality

The next challenge is in providing quality health care with a focus on patient safety. The government needs to invest to meet the costs that are involved in bringing the most innovative and effective medical technologies to India. They may seem expensive as an investment, but in the long run will be cost effective.

6 Lessons from the Philippines

Global technological advancements coming to India also require trained medical personnel such as radiology technicians, sonologists, physical therapists, medical technologists, clinical nurses, health-informatics specialists and many more to address the modern management of these lifestyle diseases. Although we see financing, organisation, data analytics and health human resource development as vital pillars of the UHC implementation in India, the most important reasons for embarking on UHC is to provide financial risk protection and remove health inequity. These are gaps that medical innovations can fill. The world is now watching India's Ayushman Bharat.

6

Lessons from the Philippines



Access to India's health-care system has not begun with the launch of Ayushman Bharat. The world's largest public health-care scheme promises to cover 400 million poor and vulnerable families. But will it have a transformative impact in a country where this is defined mostly by the private sector? Universal care is not just a matter of accessibility and affordability but also needs to be better prepared to fight the emerging problem of chronic non-communicable diseases.

As someone who has worked in the implementation of the National Health Insurance (NHI) in my own country, the Philippines, with a population of 100 million people, I'd like to share a perspective on some of the challenges.

Health expenditure

The first is the implementation of robust financial systems to health care. How does the government spend money on the health of its people?

In the Philippines, we were able to galvanise governments spending on health by financing it through all means for tobacco and alcohol. After additional tax-on tobacco and alcohol, the government increased health-care expenditure from 1.5% to 4.5% of gross domestic spending. These extra taxes added to tobacco and alcohol were "allocated" to health by legislation. The tax law allocated 80% of the additional revenue to the health-care system and the remaining 20% was allocated to tobacco farmers, who would be affected by the higher tobacco prices.

This is a unique approach and the first in the world where the law provided funds to public health, specifically towards NHI. The government's health-care financing led to larger health expenditures, which may lead to a redistribution of thousands of public hospitals in the Philippines.

For India, a better health-care spend will not only make it accessible to all but also enable the government to procure advanced medical technology from around the world or under the Indian market.

As India grows economically and socially, the shift in the disease burden is becoming more pronounced today, non-communicable diseases are posing a challenge to health-care services in the country. The government must recognise the gaps in the health system and bridge them to make modern and equitable services accessible to the poor. There is a need for primary health centres in all parts of the country to adapt to the changing disease profile. For instance, in the past, primary care meant prevention through low-cost vaccines and medicines as the burden of illness. The methodology required immunisation, oral rehydration and hand washing. But because the disease burden has shifted, primary care has changed too. Today, primary care involves weight management, blood pressure monitoring, blood sugar monitoring, ECG, blood lipoprotein, and others. To tackle this demand, state-of-the-art technology, personal monitoring devices, telemedicine, data analytics and remote care of it. There is an evident need to provide these services with modern laboratory equipment for blood tests, ultrasound, radiology, ECG, endoscopy, laparoscopic, machine point CT scanners, and so on.

Towards quality

The next challenge is in providing quality health care with a focus on patient safety. The government needs to spend to meet the need that are involved in bringing the most innovative and effective medical technologies in India. They may seem expensive as an investment, but in the long run will be cost-effective.

Global technology advancement is leading to high-end medical equipment such as robot-assisted surgery, telemedicine, physical therapists, medical technologies, clinical research, health information specialists and many more to address the modern management of these lifestyle diseases. Although we are financing, organising, data analytics and health insurance development as the pillars of the NHI implementation in India, the most important step may be utilising the NHI to provide financial risk protection and ensure health security. There are gaps that medical innovation can fill. The world is now watching India's Ayushman Bharat.

EDITORIAL DISCUSSION

LIVE

UPSC CSE – GS II

GS II ... Social Justice and Governance

Issues relating to development and management of Social Sector or Services relating to Health, Education, Human Resources.

IMPORTANT QUESTION

ESSAY/ MAINS / GD / INTERVIEWS

To make Universal Health Care a reality, "specific law attributed funding mechanism" is the need of the hour. Discuss in the context of Philippines experiment.

OR

The disease burden is shifting from communicable to non-communicable diseases. Discuss the paradigm shift required in the functioning of Primary Health Centres.





VIETNAM

Lao PDR

CAMBODIA

South China Sea

PHILIPPINES

Philippine Sea

PHILIPPINES EXPERIMENT IN PUBLIC HEALTH!



This is the first in the World, where law-attributed funds are allocated to public health, specifically towards Universal Health Care and it led to modernization of thousands of public hospitals in the Philippines.

- 1** Philippines has got a population of around 110 million people.
- 2** Philippines increased the healthcare expenditure by financing it through sin taxes for tobacco and alcohol.
- 3** The healthcare expenditure gone up from 1.8% of GDP to 4.8% of GDP, when one looks at the Government spending.
- 4** The excise taxes added to tobacco and alcohol were “attributed” to health through legislation process.
- 5** The Sin Tax Law attributed 85% of the additional excise tax revenues to the healthcare system and remaining 15% attributed to tobacco farmers.



SHIFT IN THE PRIORITIES!

- 1 Disease burden is shifting today and non-communicable diseases are posing a challenge to healthcare services,
- 2 The Government must recognise the gaps in the health system. It should bridge them to make modern and equitable services accessible to the poor.
- 3 Primary Health Centres should adapt to the changing disease profile.

PRIMARY HEALTH CENTRES ... A PARADIGM SHIFT REQUIRED

WHAT IS THE EMPHASIS TILL NOW?

- 1 Sanitation and Hygiene were the major causes of illness
- 2 The methodology required immunisation, oral rehydration and hand washing etc.



WHAT SHOULD BE THE EMPHASIS FROM NOW ONWARDS?

- 1
 - Weight management
 - Blood Pressure monitoring
 - Blood Sugar monitoring
 - ECG
 - Blood Chemistry and others.
- 2 There is a need to provide these centres with modern laboratory equipment for blood tests. Ultra-sound, radiology, ECG, endoscopy etc.

India needs modern technology, personal monitoring devices, telemedicine, data analytics and extensive use of IT.

PRIMARY HEALTH CENTRES ... A PARADIGM SHIFT REQUIRED

WHAT IS THE EMPHASIS
TILL NOW?



IMMUNIZATION



ORAL REHYDRATION



HAND WASHING

WHAT SHOULD BE THE EMPHASIS
FROM NOW ONWARDS?



WEIGHT
MANAGEMENT



BLOOD PRESSURE
MONITORING



BLOOD SUGAR
MONITORING



ECG

WHAT ARE THE FOUR PILLARS?

1

FINANCING



2

DEVELOPING ORGANIZATION



3

DATA ANALYTICS



4

**HEALTH HUMAN RESOURCE
DEVELOPMENT**





LIFESTYLE DISEASES ... DEVELOPMENT OF MANPOWER

The need of the hour is

- Radiology technicians
- Sonologists
- Physical therapists
- Medical technologists
- Clinical nurses
- Health-informatics specialists and many more.

*The World is now watching
India's Ayushman Bharat!*

WE EXPRESS OUR
SINCERE THANKS
FOR VIEWING THIS VIDEO

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